

The Importance of Diagnosing Premenstrual Syndrome

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Introduction

In the 1950s, doctors were urged to look afresh at their chronic patients to ensure that they were not suffering from hidden depression which would respond to the newly discovered antidepressants. Now, with our improved understanding of premenstrual syndrome (PMS), there is a similar need to take a new look at those chronic problem patients who are women in their menstruating years. We need to see whether they are victims of PMS who are suffering needlessly when they could be given the benefit of the highly successful treatment with natural progesterone.

Recently, premenstrual syndrome has been given legal recognition as a cause of diminished responsibility in murder, and as a mitigating factor in crimes of arson, assault, shoplifting and disorderly conduct. However, the law requires more than the mere statement that the woman was in her premenstruum at the time of her crime. Evidence must be produced to show that the symptoms recur regularly each month, and that treatment is likely to be successful and ensure that she will not repeat the offence.

Characteristics of PMS

Premenstrual syndrome is a disease of progesterone deficiency. The symptoms of PMS will therefore only occur in the second half of the menstrual cycle when adequate amounts of progesterone in the peripheral blood are required. After menstruation and until ovulation, progesterone is not present in the peripheral blood and so PMS symptoms will not occur. Therefore it follows that for the diagnosis of PMS it is essential to show that the timing of symptoms is limited to the premenstruum or early menstruation, with a complete absence of symptoms after menstruation. Diagnosis is best achieved by use of a menstrual chart on which the woman marks with an 'M' the dates of menstruation (or with a 'P' the dates of her period), and then uses other symbols for her symptoms, for example 'X' for the days of quarrels, alcoholic bouts or asthma attacks, 'T' for tiredness, 'F' for forgetful days and 'H' for headaches. Each woman can select symbols for her most troublesome symptoms. Charts are obtainable free from Cox-Continental Ltd, Brookside Avenue, Rustington, Sussex. Diagnosis rests on ensuring that there is a complete absence of symptoms during the postmenstruum. With a menstrual chart this diagnosis can be made at a glance, in fact even a ten year old child can recognize those charts which have no symbols for at least seven days after the 'M's.

Interviews with husbands and parents are invaluable, as those closest to the PMS sufferer will know best how she changes from the happy, charming individual which she is on most days into the lazy, irritable and unpredictable female she becomes as menstruation draws near.

Retrospective diagnosis

In order to obtain a retrospective diagnosis it may be helpful to conduct searches elsewhere. Looking up the school register may reveal a monthly pattern of school truancy, as occurred in one teenager who was frightened to go to school on those days when his mother hit the bottle. Hospital records will show the precise dates of overdoses and self-mutilation attempts. The works register may be consulted to show cyclical absences, and police documents may display regular patterns of assault or drunk and disorderly behaviour. Even prison files may exhibit evidence of regular monthly episodes of bizarre behaviour occurring while in custody. All such information is valuable in building up positive evidence that the

woman has symptoms at menstruation with normal behaviour at other times.

Other diagnostic pointers worth bearing in mind are the frequency with which the onset of PMS, and the occasions of increased severity, occur at times of upheaval of the menstrual hormones. These times include puberty, after pregnancy, during or after stopping the pill, after a spell of amenorrhoea such as occurs in anorexia nervosa, and following sterilization. Women with PMS usually have normal, pain-free and regular menstruation, and never experience the spasms of lower abdominal colicky pains so typical of spasmodic dysmenorrhoea.

Women with PMS are usually the ones who develop side effects on the pill, complaining that it causes headaches, depression or weight gain. They change their brand of pill and finally resort to some other contraceptive method. There is a high incidence of about nine women in every ten who develop PMS after certain complications associated with pregnancy. These include pre-eclampsia severe enough to require hospital admission in the antenatal period, or postnatal depression severe enough to need antidepressant treatment in the puerperium. There may also be a history of a threatened miscarriage in the early months with a successful outcome of pregnancy.

Body chemistry

During the premenstruum, women with PMS experience a rise in glucose tolerance which results in food cravings and binges. The frequent weight gain and bloatedness in the premenstruum brings with it a desire to diet, so causing problems of intermittent feasting and famine. When PMS sufferers go too long without food, their blood glucose level reaches baseline and there is a compensating spurt of adrenalin to release glucose from the body's store. It is this sudden release of adrenalin which is responsible for many of the premenstrual symptoms such as panic attacks, explosive outbursts, uncontrollable aggression and migraine. After all, adrenalin is recognized as the hormone of fear, fight and flight. Careful enquiry will elicit the fact that most instances of baby battering and assault occur after a long food gap (five hours in the day or thirteen hours overnight). The mother who misses breakfast is most likely to be irritable at lunchtime, and the housewife who forgets her midday meal will be aggressive when her husband returns from work in the evening.

Alcohol tolerance varies over the menstrual cycle and those with PMS may find that during the premenstruum they are unable to tolerate their normal alcohol intake without signs of intoxication. The problem is increased by their urges to take alcohol at this time to overcome their premenstrual depression and lethargy. Thus during the premenstruum, at a time of lowered self-discipline, lowered self-control, alcohol urges and increased tendency to intoxication, is it any wonder that charges of drunk and disorderly occur with such frequency on the woman's charge sheets? Yet these same women may have no difficulty in totally abstaining from alcohol at other times of the menstrual cycle.

Conclusion

One in ten of all menstruating women are estimated to suffer from PMS severe enough to require treatment. Diagnosis is simple, but the responsibility of referring possible sufferers to their general practitioner frequently rests with health visitors, and they should not fail in this task.

Further reading

Dalton K. Once a month. London: Fontana, 1978.
Dalton K. Premenstrual syndrome and progesterone therapy. London: Heinemann Medical, 1977.

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